

إسم المريض ربيعاً : نادية محمد جمال الدين الجزار
المحافظة : القاهرة الحي / المدينة : دار السلام
رقم البطاقة : 26310241701783 رقم التليفون : 01118196942
العمر(تاريخ الميلاد) : 24-10-1963 المهنة : ربه منزل
العنوان : 2ش محمود سليمان
تاريخ تسجيل الإستمارة : 29-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: نادية محمد جمال الدين الجزار National ID: 26310241701783	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bilateral mammogram and breast ultrasound
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





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MDT FORM 6 / 11 / 2022 G



Patient Name : الأستاذة / نادية محمد جمال
Patient No. : 26310241701783
Path No. : 265/2022
Sex : Female
Age : 58 Y
Referring Doc. :

Request No. : 13001759
Request Date : 31/01/2022
Verification Date : 01/03/2022
Financial Category : أورام نادر - أورام
Department : Out Patient
IHC No. :
Registered by : .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Rt & Lt breast mass

Nature of the specimen: tru cut

Gross Examination:

) multiple gray white tissue cores up to 2cm, totally submitted

) multiple gray white tissue cores up to 1.4cm, totally submitted

Sections prepared from paraffin block for Immune- staining using Bench Mark ULTRA system for ER, R, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9)

Microscopic examination:

Histological type: Bilateral IDC

Grade (differentiation + nuclear pleomorphism + mitosis): GII(2+2+1)

Angiolymphatic invasion: NO

Ductal carcinoma in situ: positive

Tumor extent

Number: N/A

Size: N/A

Skin involvement: N/A

Lymph node involvement: N/A

Resection Margins: N/A

Histologic response to therapy: N/A

Additional pathologic findings: N/A

Specimen (1):

Examination of ER stained slide show intense nuclear staining in 70% of the tumor cells.

Examination of PR stained slide show moderate nuclear staining in 90% of the tumor cells.

Examination of HER2 stained slide show no membranous staining in tumor cells.

Verified by :

walaa ghoniem

Printing Date : 01/03/2022 09:55:12

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Ministry of Health - State of Palestine
Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - مختبر الأمراض النسائية

Patient Name : 13003630 / 17/09/2022
Patient No. : 26310241701783
Path No. : 2734/22
Sex : Female
Age : 58 Y
Referring Doc :

Request No. : 13003630
Request Date : 17/09/2022
Verification Date : 27/10/2022
Financial Category : مريض - Out Patient
Department :
Registered by : Ali Mostafa

Pathology Report

Clinical Data

Bilateral breast cancer, Post NACT

Nature of the specimen:
Bilateral MRM

Gross Examination:

- * Right MRM measured 32x29x5 cm with covering skin measured 20x12 cm, covering skin, nipple & areola were unremarkable grossly, sectioning revealed an ill-defined tumor bed site, 6x3x1 cm, located 3 cm from deep margin, 3.5 cm from the skin. Axillary dissection revealed 17 nodules up to 2 cm in diameter. Rest of breast showed scattered foci of grayish white rubbery area.
- * Lt MRM measured 35x29x6 cm with covering skin measured 35x29 cm, covering skin, nipple & areola were unremarkable grossly, sectioning revealed an ill-defined tumor bed site, 6x5x4 cm, located 1 cm from deep margin, 1.5 cm from the skin. Axillary dissection revealed 17 nodules up to 1 cm in diameter. Rest of breast showed scattered foci of grayish white rubbery area.

Microscopic examination

Right MAM:

- * Total tumor regression.
- * Complete pathological response.
- * Skin involvement: Free.
- * Lymph node involvement: No (0/17).
- * Resection Margins: Free.
- * Free nipple & areola.
- * Histologic response to therapy: Complete pathological response.
- * Additional pathologic findings: Hyalinosis, Necrosis, Fibrosis in surrounding breast.

Verified by

Dr. Marwa Mahmoud

Printing Date: 19/10/2022 11:37:30

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POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED:-

1. Chest:

- Left breast UOQ shows Irregular mass measuring about 4.9x4.9 cm with overlying thickened skin.
- Left axillary level I, II and III amalgamated enlarged lymph nodes are seen the largest measures about 27x26 mm.
- Left supraclavicular and left posterior cervical enlarged lymph nodes are seen the largest is at the latter measuring about 17x13 mm.
- No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right thyroid hypodense nodule is seen measuring about 13 mm.

2. Abdomen and pelvis:

- The liver is enlarged showing homogenous CT texture. Left hepatic lobe intensely enhancing focal lesion is seen at segment III measuring about 15x12 mm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No Intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus is seen with multiple masses likely uterine fibroids.
- No abdominal or pelvic masses or collections.
- Skeletal examination revealed:
 - Dorsolumbar spondylotic changes are seen.
 - 1st degree L4/5 degenerative spondylolisthesis.
 - D12 vertebral body tiny sclerotic bone lesion is seen likely bone island.

OPINION:

- Left breast mass with left axillary level I, II and III as well as left supraclavicular and left posterior cervical enlarged lymph nodes.
- Left hepatic lobe intensely enhancing focal lesion likely hemangioma for local US confirmation.

Dr. Reham Anwar, MSc, FEBR

Dr. Amy Farouk MD

DATE: 10/03/2012
CASE NO: 1002301

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- 59-year-old lady presented with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- No evidence of bone secondaries could be seen.
- Degenerative changes are noticed at different levels of the spine.
- Osteoarthritic features are observed at the both knees.

IMPRESSION

- The skeleton is free from metastases.

*Much obliged,
Dr. Alaa Nagh, MD
Nuclear Medicine*